

OpenTumorBoard

Real multidisciplinary tumor boards, scored as two tasks

REAL TUMOR BOARD



16 min of recorded discussion

52-year-old man.

Gross hematuria and increased lower urinary tract symptoms at initial presentation.

8 turns · 6 specialists · 13 questions

219	187	611	16,215
videos	hours	cases	questions

HOW IT IS BUILT

ASR · diarization · case segmentation · slide extraction · role inference · alignment · QA generation

nine stages, detailed in Fig. 2a

WHAT THE MODEL IS GIVEN

identical for both tasks

Case summary

High-risk localized prostate cancer. Initial biopsy demonstrated Gleason score 5+5=10, Grade Group 5 adenocarcinoma. Subs...

8 slide captions

Slide summarizing Case #1, a 52-year-old man's presentation, workup findings, imaging, biopsy results, and relevant medic...

no reference answer, no transcript, no tools

TASK 2 Expert QA

answer as the named specialist

role: medical oncologist

findings interpretation

Q What was the clinical significance of a Ki-67 of about 80% despite negative neuroendocrine marker staining?

Reference An approximately 80% Ki-67 raised concern that the patient had micrometastatic disease, even though neuroendocrine marker staining was negative.

verbatim from the real expert · scored by clinical equivalence

TASK 1 Tumor Board Simulation

run the whole board

Generate the multi-specialist discussion, then the plan

real board: 6 roles over 8 turns

moderator

pathologist

radiologist

medical oncologist

surgeon

radiation oncologist

Reference conclusion

The multidisciplinary review supported continuation of androgen deprivation therapy and radiation therapy, with the addition of platinum-etoposide chemotherapy because of the biopsy findings concerning for small cell/neuroendocrine features. Four cycles of cisplatin and etoposide were administered from December through March. Given persistent significant lower urinary tract symptoms, absence of metastatic disease, and negative cystoscopy for trigonal involvement, the subsequent course included direction toward surgical management.

distilled from the real board's decision · scored on role coverage, clinical accuracy and conclusion alignment